

Case Manager General Responsibilities

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Case Management

Overview

Case management is a collaborative process of assessment, planning, facilitation, care coordination, evaluation, and patient advocacy. The goal of case management is to ensure that patients have a safe, effective care plan that promotes optimal patient outcomes utilizing appropriate available resources.(1)(2) Case managers create a case management plan to address the immediate and ongoing needs of the patient and continue to evaluate the plan to measurable goals and objectives. The case management plan is based on evidence-based guidelines, approved clinical pathways, or protocols. Case management planning is done in collaboration with the patient, caregivers, providers, and payers.(3) Case managers are employed by provider offices, health plans, hospitals, recovery facilities, home health agencies, or directly by patients or their families.(3)(4) Geriatric case managers (GCM), an evolving profession, provide guidance and resources for older adults and their caregivers around chronic illness or disabilities, helping them navigate through the healthcare continuum and improving their quality of life.(5)

Assessments for case management planning include: current clinical and educational needs, home safety, ADL or IADL capabilities, psychosocial status, social determinants of health, and support to manage chronic conditions. Implementation and evaluating the plan requires the case manager to monitor the patient's status with quality measures and outcome results, and implement processes or plan adjustments to reach the plan goals.(3)(4) When variance from the plan exists, the case manager recommends possible solutions, facilitates plan adjustments, and continues to evaluate patient progress to promote patient safety and progression through the care continuum.

Case managers may employ a variety of tools (eg, action plans, transition of care checklists) to facilitate their roles and responsibilities.(3)(4) They also employ behavior modification techniques, advocacy services, change agent services, guidance for individuals through the healthcare process, and track processes and outcomes. This includes providing education and motivation to decrease the risks of illness, injury, or disease, and serving as a liaison between the patient and providers.(3)(4)

The Case Management General Responsibility tool contains a list of general activities of the case manager at each level of care. It may be used as a checklist to augment the specific information that appears in each guideline, to develop additional case management tools, or to provide a systematic review of responsibilities. The case manager should adapt these responsibilities to the needs and protocols of each agency or organization and exercise judgment to determine which of these services are needed. The case manager should act within the scope of their professional license.(2)(4)

Additional supporting assessment tools include:

- Discharge Planning Tool  SR
- Home Safety Assessment  SR
- Activities of Daily Living (ADL) and Instrumental Activities of Daily Living (IADL) Assessment  SR
- Psychosocial Assessment  SR
- Social Determinants of Health Screening Tool  SR
- Teach Back Tool  SR
- Post-Acute Care Follow-Up Call Tool  SR

Office or Rapid Treatment Site

- Obtain authorization for care, if needed.(6)
- Complete assessments, including(7):
 - History and physical examination
 - Psychosocial assessment, including:
 - Barriers to adherence (eg, health literacy, cultural expectations)
 - Behavioral motivation
 - Financial needs and social support system
 - Social determinants of health
 - Functional assessment
 - Diagnostic testing
 - Home safety, considering needs, capabilities, and altered mobility
 - Patient knowledge of health or disease process and medication(8)
- Identify problems and establish goals of case management plan in collaboration with patient and caregiver.(1)
- Implement and evaluate case management plan, which may include:
 - Facilitate referrals to care providers, condition management programs, rehabilitation programs, or psychiatric services.
 - Provide education on medical condition (eg, medication, complications, ongoing management).(9)
 - Begin transition of care planning as indicated by diagnosis or condition of patient.(10)(11)
 - Coordinate development of treatment goals and plans with patient and multiple professionals, including:
 - Primary care provider
 - Specialty physician providers
 - Therapists
 - Home healthcare staff
 - Recovery facility staff
 - Ensure timely referral to appropriate community resources and applications for public funding.
 - Ensure transition of care plan has been implemented in collaboration with patient and caregiver, including(12):
 - Notification has been provided to all appropriate members of healthcare team.
 - Identify patient's access to, type of, and ability to navigate health insurance and prescription benefits.
 - Medication reconciliation is complete.(13)
 - Ensure that patient or caregiver knows how to reach appropriate contact for assistance if any complications arise after discharge.(9)
 - Facilitate admission to appropriate level of care, if indicated.
 - Acute care hospital readmission prevention(14)(15)
 - Provide self-management teaching on appropriate education and health literacy level.(8)
 - Complete medication reconciliation.
 - Improve communication between providers and patients/families.
 - Coordinate any follow-up appointments with specialists as needed.
 - Identify patient at risk for readmission (eg, adherence barriers, high severity of illness, self-management inabilities).
 - Provide follow-up phone calls postdischarge to address medication, plan of care instructions, and ensure attendance with follow-up appointments.

Health Plan

- Obtain authorization or consent for care, if needed.(6)
- Complete assessments, including(1):
 - Current treatment plan
 - Health status
 - Psychosocial status
 - Social determinants of health
 - Benefit structure
 - Patient knowledge of health or disease process and medication
 - Current barriers to treatment plan
 - Readmission risk
- Identify problems, and establish goals of case management plan in collaboration with patient and caregiver.(1)
- Implement and evaluate case management plan.
- Facilitate interventions.
 - Acquire authorization for services available within benefit terms and health plan operational rules.(6)
 - Review requested services in relation to evidence-based care guidelines.
 - Communicate treatment plan to all members of interdisciplinary treatment team along care continuum.(12)
 - Provide education to patient and caregivers regarding(9):
 - Health status
 - Treatment plan
 - Health system navigation

- Benefit terms
- Costs of healthcare services
- Evidence of quality of healthcare services
- Community resources
- Engage and empower patient to take responsibility for health activities and health status.(8)
- Advocate to overcome any barriers to necessary care.(6)
- Plan to continue patient surveillance, including identification of readmission risk factors.
 - Acute care hospital readmission prevention techniques include(6)(15):
 - Provide self-management teaching on appropriate education level.(8)
 - Complete medication reconciliation.
 - Coordinate any follow-up appointments with specialists, as needed.
 - Assess and address risk factors; examples include(14):
 - Completion of diagnostic workup ordered after discharge
 - Adherence barriers
 - High severity of illness
 - Debility level
 - Self-management inabilities
 - Advanced age
 - Two or more admissions prior to current admission
 - End-of-life care
- Measure outcomes of case management interventions to confirm program effectiveness, and modify plan as needed, including:
 - Health status
 - Costs associated with health services
 - Patient satisfaction
 - Case management program
 - Health status
 - Health system
 - Provider satisfaction
 - Quality measures

Hospital

- Obtain authorization or consent for care, if needed.(6)
- Determine and obtain authorization for continued stay, if indicated.
- Review hospital stay care plan.
- Complete assessments, including(1):
 - Current treatment plan
 - Health status
 - Functional status, including ability to perform ADL and IADL safely, with or without assistance
 - Psychosocial status (eg, financial needs, support system)
 - Social determinants of health
 - Patient knowledge of health or disease process and medication
 - Current barriers to treatment plan
 - Readmission risk
 - Adverse event risk
 - Home safety, if home discharge anticipated(16)
 - Key issues and barriers to discharge
 - Benefit structure
- Identify problems and establish goals of case management plan in collaboration with patient and caregiver.(1)
- Implement and evaluate case management plan.
- Facilitate interventions by monitoring daily progress, ensuring that:
 - Patient health status monitored in relation to Optimal Recovery Course (eg, applicable recovery milestones attained, interventions in place to avoid variances, and variances to optimal recovery identified promptly)
 - Physical status monitored
 - Vital signs appropriate to condition
 - Resolution of signs and symptoms related to condition or procedure
 - Emergence or resolution of complications from condition or procedure
 - Comorbidities managed
 - Clinical issues managed
 - Appropriate testing is completed.
 - Arrangements for needed consultation are made as soon as possible.
 - Ambulation and rehabilitation begin as early as possible.
 - Preventive measures for common complications (eg, skin integrity impairment, DVT prophylaxis) are in place.

- Facilitate transition from parenteral to oral medication or nutrition, when indicated.
- Perioperative antibiotics are administered in a timely manner, when indicated.
- Psychosocial status issues addressed, including:
 - Age
 - Psychological comorbidity
 - Caregiver ability and availability
 - Impact on lifestyle, beliefs, and family and social roles
 - Expectations for resumption of functional ability
 - Need for support and referrals
 - Behavioral or cognitive problems
 - Barriers to discharge (eg, financial, transportation, or health literacy) addressed
- Develop transition of care plan(12)(17)
 - Coordinate development of treatment goals and plans with patient and multiple professionals, including(18):
 - Primary care provider
 - Specialty physician providers
 - Therapists
 - Home healthcare staff
 - Recovery facility staff
 - Facilitate goal setting with patient and/or caregiver, including promotion of self-management.
 - Facilitate decision about next level of care, including(11):
 - Identify appropriate level of post-acute care (eg, appropriate provider and availability of services).
 - Present options and identify preferences of patient or caregiver for post-acute care.
 - Coordinate financial resources and payers.
 - Ensure timely referral to appropriate community resources and applications for public funding.
 - Initiate medication reconciliation pre-discharge and post-discharge.(13)
- Facilitate transition to next level of care.
 - Arrange equipment (eg, DME) that may be needed at facility or in home environment.
 - Ensure follow-up care and appointment(s) are made and transportation is available.
 - Facilitate referrals to care providers, condition management programs, rehabilitation programs, or psychiatric treatment.
 - Implement transition of care plan in collaboration with patient.(11)(18)
 - Communicate plan to all members of healthcare team.
 - Identify patient's access to, type of, and ability to navigate health insurance and prescription benefits.
 - Identify available community and health resources.
 - Complete medication reconciliation.
 - Identify patient/caregiver education materials to be given at time of discharge.
 - Educate patient and caregiver to support self-management, independent living, and ADL.(9)
 - Coordinate any follow-up appointments with specialists, as needed.
 - Ensure patient and caregiver know how to reach appropriate contact for assistance if any complications arise after discharge.(9)
 - Ensure patient or caregiver understand importance of presenting personal medication list to all providers at each care transition, including all provider appointments.(9)
 - Identify and address readmission risk factors (eg, adherence barriers, high severity of illness, self-management inabilities).
 - Identify and address potential adverse event risk factors post discharge (eg, new or worsening symptoms, laboratory test abnormalities, adverse drug events, falls, or hospital-acquired infections).
 - Arrange follow-up call with patient/caregiver shortly after discharge.
- Provide regular communication to providers, patient, family, caregiver, and payer.
- Plan to continue patient surveillance.(6)
- Complete documentation and legal reporting for existing rules and regulations.

Recovery Facility

- Obtain authorization or consent for care, if needed.(6)
- Ensure development of care plan for recovery facility stay.
- Complete assessments, including:
 - Assess psychosocial status.
 - Age
 - Depression, anxiety, or psychological comorbidity
 - Caregiver availability and support system
 - Impact on lifestyle, beliefs, and family and social roles
 - Ability to return to usual activities (eg, hobbies, work, sports)
 - Expectations for resumption of functional ability
 - Need for support and referrals

- Behavioral problems
- Barriers to adherence and behavioral motivation
- Impaired cognition
- Patient knowledge of health or disease process and medication
- Assess social determinants of health (eg, financial needs, food insecurity).(6)
- Identify problems and establish goals of case management plan in collaboration with patient and caregiver.(1)
- Implement and evaluate case management plan, which may include:
 - Identify key issues and barriers to discharge, and ensure that:
 - Appropriate testing is completed.
 - Arrangements for needed consultation are made.
 - Ambulation and rehabilitation begin as early as possible.
 - Preventive measures for skin integrity, DVT prophylaxis, nutrition, and delirium are in place.
 - Financial needs and social support system are addressed.
 - Monitor progress, including:
 - Physical status
 - Vital signs appropriate to condition
 - Resolution of signs and symptoms related to condition or procedure
 - Emergence or resolution of complications from condition or procedure
 - Comorbidities managed
 - Pain management
 - Wound management
 - Drain management
 - Catheter management
 - Respiratory care management
 - Nutrition management
 - Medication management
 - Ensure timely transition from parenteral to oral medication.
 - Assess for drug interactions and use of cost-effective medication.
 - Develop transition of care plan(10)(12)
 - Assess ability to return home and whether adequate caregiver or assistance is available.
 - Assess need for guardian or conservator in order for patient to move to next level of care.
 - Assess necessity and availability of home healthcare support or alternate services and DME.
 - Perform or initiate home safety assessment, including identification of architectural barriers (eg, stairs, narrow or obstructed doorways) that may compromise patient's ability to access essential areas of home.
 - If return home is not possible, determine availability of long-term care or community-based residence (eg, assisted living) and available funding.
 - Coordinate development of treatment goals and plans with patient and multiple professionals, including:
 - Primary care provider
 - Specialty physician providers
 - Therapists
 - Home healthcare staff
 - Recovery facility staff
 - Ensure timely referral to appropriate community resources and applications for public funding.
 - Educate patient and caregiver.
 - Provide patient/caregiver education materials.
 - Facilitate transition to next level of care.(11)(17)
 - Arrange equipment (eg, DME) that may be needed at facility or in home environment.
 - Ensure follow-up care and appointment(s) are made and transportation is available.
 - Facilitate referrals to care providers, condition management programs, rehabilitation programs, or psychiatric treatment.
 - Implement transition of care plan in collaboration with patient.
 - Communicate plan to all members of healthcare team.
 - Identify patient's access to, type of, and ability to navigate health insurance and prescription benefits.
 - Complete medication reconciliation of discharge medications and medications taken at home prior to admission to SNF.
 - Provide patient and caregiver education materials and written instructions.
 - Educate patient and caregiver.(19)
 - Coordinate any follow-up appointments with specialists, as needed.
 - Ensure patient and caregiver know how to reach appropriate contact for assistance if any complications arise after discharge.(9)
 - Ensure patient or caregiver understand importance of presenting personal medication list to all providers at each care transition, including all provider appointments.(9)

- Identify and address readmission risk factors (eg, adherence barriers, high severity of illness, self-management inabilities).
- Identify and address potential adverse event risk factors post discharge (eg, new or worsening symptoms, laboratory test abnormalities, adverse drug events, falls, hospital-acquired infections).
- Arrange follow-up call with patient and caregiver shortly after discharge.(19)
- Regularly communicate with providers, patient, family, caregiver, and payer.
- Plan to continue patient surveillance.
- Complete documentation and legal reporting for existing rules and regulations.
- Facilitate admission to home, long-term care facility, or community-based residence, with or without home healthcare, as needed.

Home Healthcare

- Obtain authorization or consent for care, if needed.(6)
- Complete assessments, including:
 - Assess home safety, considering needs, capabilities, and altered mobility of patient.(16)
 - Ability to perform ADL and IADL safely, with or without assistance
 - In-home mobility, including gait, strength, and endurance
 - Need for DME or assistive devices; arrangement of referral if needed
 - Risk for falls and whether safety teaching is needed
 - Assess psychosocial status.
 - Age
 - Depression, anxiety, or psychological comorbidity
 - Caregiver availability and support system
 - Impact on lifestyle, beliefs, and family and social roles
 - Ability to return to usual activities (eg, hobbies, work, sports)
 - Expectations for resumption of functional ability
 - Need for support and referrals
 - Behavioral problems
 - Barriers to adherence and behavioral motivation
 - Impaired cognition
 - Patient knowledge of health or disease process and medication
 - Assess social determinants of health (eg, financial needs, food insecurity).(6)
 - Assess presence and ability of caregiver to meet patient's needs.(9)
- Identify problems, and establish goals of case management plan in collaboration with patient and caregiver.(1)
- Implement and evaluate case management plan, which may include:
 - Continue rehabilitation as appropriate.
 - Monitor progress, including:
 - Physical status
 - Vital signs appropriate to condition
 - Resolution of signs and symptoms related to condition or procedure
 - Emergence or resolution of complications from condition or procedure
 - Comorbidities managed
 - Pain management
 - Wound management
 - Drain management
 - Catheter management
 - Respiratory care management
 - Nutrition management
 - Medication management
 - Manage IV therapy, if indicated.
 - Assess for drug interactions and use of cost-effective medication.
 - Develop transition of care plan(12)(17)
 - Coordinate development of treatment goals and plans with patient and multiple professionals, including:
 - Primary care provider
 - Specialty physician providers
 - Therapists
 - Home healthcare staff
 - Ensure follow-up care and appointment(s) are made and transportation is available.
 - Facilitate referrals to care providers, condition management programs, rehabilitation programs, or psychiatric treatment.
 - Implement transition of care plan in collaboration with patient, and notify all appropriate members of healthcare team.
 - Ensure timely referral to appropriate community resources and applications for public funding.
 - Educate patient and caregiver.
- Regularly communicate with providers, patient, family, caregiver, and payer.

- Plan to continue patient surveillance.
- Complete documentation and legal reporting for existing rules and regulations.

Patient and Caregiver Education

- Provide patient and caregiver education as appropriate for disease or condition.
- See Care Management Tool, Education: Patient, Family, and Caregiver  SR.

Resources

- Case Management Society of America (CMSA): www.cmsa.org
 - American Case Management Association (ACMA): www.acmaweb.org
 - National Transitions of Care Coalition (NTOCC): www.ntocc.org
 - Population Health Alliance (PHA): www.populationhealthalliance.org
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